

SECTION I

An Overview of Internal Family Systems Therapy

THE CORE ASSUMPTIONS OF IFS

The IFS model of psychotherapy, developed by Richard Schwartz, is based on three assumptions. First, everyone has many *parts*, also known as subpersonalities, who engage with one another in their own internal community and operate much the way families and larger groups operate (Schwartz & Sweezy, 2020). Parts are normal. Having *many* parts is normal. They manifest in a developmental sequence throughout life, showing up at different ages and in different places in the body. Each part expresses its feelings and beliefs in its own way.

Second, contrary to initial appearances, the psyche's inner players are not random or unrelated. They are a system and they relate to one another in patterned ways, forming alliances and polarizing. When one part (or team of parts) becomes extreme in an effort to be safe, another part steps in to restore balance by moving in a different direction. A power struggle ensues and each side becomes more extreme, which reinforces emotional pain. In IFS, we therefore focus directly on polarizations between opposing parts as we treat clients with addictions.

Third, every person has what Schwartz calls a *Self* in addition to having parts. Schwartz landed on this word because his clients used it to describe what they felt when their parts cooperated rather than vying for control. They felt calmer, more accepting, and more spacious, and would say, "I feel more like myself now." To distinguish this concept from more mundane uses of the word, Schwartz capitalized the "S" in *Self* in his writing. In IFS, we see the Self as an undamageable inborn resource. When clients are connected with their Self, they embody qualities like curiosity, calm, courage, compassion, creativity, connection, confidence, and clarity, which Schwartz dubbed the 8 C's of Self-energy. Since a client's growing capacity to make healthy decisions is especially important in addictions treatment, this manual adds one more C to the list: choice.

EIGHT C'S OF SELF-ENERGY

Calm

Creative

Connected

Compassionate

Confident

Clarity

Courageous

Curiosity

+ Choice (9th C)

PSYCHIC MULTIPLICITY IS NOT A PATHOLOGY

In the internal family system, parts take on roles that can be divided into three distinct categories: managers, firefighters, and exiles. Managers are controlling parts who are proactive, purposeful, and future-oriented. In contrast, firefighters are reactive, impulsive, compulsive, and present-oriented. Although they act in different ways, managers and firefighters share the goal of protecting the sensitive, vulnerable parts—the exiles who can overwhelm the whole system with their emotional pain—and keeping them out of everyday consciousness.

Although these three categories of parts can be extreme, they are all integral to the psyche, and each has an indispensable role to play in our lives. The open-hearted, vulnerable exiles bring us curiosity, spontaneity, and joy; the managers make sure we function well and get things done; and the firefighters ensure that we take time to relax and have pleasure. Once the internal system trusts the Self and is willing to follow its lead, clients discover the benefit of welcoming all parts and recognize the strengths of each.

MANAGERS: PROACTIVE AND FUTURE-FOCUSED

When it comes to managers, the name says it all. This category of protectors does everything in its power to protect the system by planning ahead for any situations that could result in emotional pain. Toward this end, they promote two different goals. First, they take care of basics, like holding down a job, providing a place to live, and tending to social relationships. Second, they promote progress and personal success. For example, a striving manager might motivate a client to improve at work, get better grades, or learn new parenting skills at home. Alternatively, a judging manager might warn a client about being too angry or procrastinating too much.

Common manager behaviors include being task-oriented, rational, directive, discerning, judicious, cautious, critical, organized, analytical, serious, caretaking, concerned, thoughtful, approval-seeking, vigilant, and attentive, among others. Manager behaviors—even criticism—are designed to ensure at least a reasonable level of stability in work, living conditions, and primary relationships. In a balanced system, clients get a solid sense of confidence, satisfaction, and pride from manager-led accomplishments. Managers typically want the client to look as positive as possible: smart, likable, kind, prepared, and successful in whatever realms they choose. Since these behaviors tend to be accepted and reinforced by our larger social culture, it's easy for us to identify with most of the views of our managers.

However, some managers need to make great efforts to protect us during particularly painful moments. When clients are struggling with addictive processes, their managers are typically demanding, controlling, obsessive, and even vicious. What was once an accepted behavior in society might now be a strict order. For instance, the simple desire to do better may become a harsh demand to be flawless, and anything less than perfection is deemed worthless. These managers have no patience for mistakes and little tolerance for fear or sensitivity. Even when clients are functioning well at work and home, these parts are never satisfied. They unleash internal diatribes that fill the client with contempt, moralizing, and blame. On the whole, their intentions are positive, but their behavior tops off an inner reservoir of shamefulness and mobilizes reactive firefighter parts, who are always prepared to bring relief with substances or other options.

COMMON BEHAVIORS OF MANAGER PARTS IN ADDICTIVE SYSTEMS

- **Blaming critic:** Attacks firefighters with hostility and contempt

- **Shaming judge:** Evaluates firefighter behaviors as immoral or bad
- **Perfectionist:** Is terrified of mistakes and assumes only one way is correct
- **Logical rationalist:** Analyzes what to do by considering facts, not emotions
- **Intellectual:** Prefers talking about problems instead of taking action
- **Striver:** Is highly competitive and demanding with self and others
- **Rescuer:** Prevents other people from experiencing the natural consequences of their risk-taking behavior
- **Fixer:** Assumes personal responsibility for other people's actions or problems
- **Caretaker:** Has excessive concern about other people's feelings
- **Over-giver:** Is generous to a fault and struggles with boundaries
- **Know-it-all:** Is always right and wants things done their way
- **Controller:** Attacks firefighters and anyone else who doesn't conform to norms

COMMON MANAGER CHARACTERISTICS IN ADDICTIVE SYSTEMS

- They are responsible, compulsive workers who get tasks completed and value being productive and good. (The part may say: *This is the real me.*)
- They care about appearances and make efforts to seem legitimate, valuable, and normal.
- They are chronically anxious and vigilant. They can't relax or trust anyone—not the Self, firefighters, nor exiles—so they are very self-reliant.
- They work overtime to prevent exiles and firefighters from taking over by engaging in hostile, shaming, and blaming behavior.
- They operate from the neck up and minimize or dismiss feelings.

COMMON MANAGER FEARS

- Fear of the chaos and unpredictability created when firefighters are in action
- Fear that feelings of shame and worthlessness will flood the internal system, especially after firefighter activity
- Fear that the Self is not strong enough to help firefighters, often doubting that the Self exists
- Fear that secrets, old memories, and painful emotional wounds will be exposed and reinjured
- Fear that nothing will change, that firefighters cannot change their behavior, and that the underlying isolation, grief, and shame cannot be healed

FIREFIGHTERS: REACTIVE AND PRESENT-FOCUSED

The firefighters in your town or city are brave, committed, and ready for action. They do whatever it takes to save others, including risking their own lives. This is the image Schwartz had in mind when he labeled the second category of protectors. Also known as distractors or soothers, firefighter parts are reactive. They leap into action when the emotional pain of feeling worthless and unlovable breaks into consciousness, doing whatever it takes to distract the mind and soothe the body.

Some firefighter behaviors tend to be viewed as more socially acceptable. These include snacking, eating sweets, having a few drinks, eating cannabis edibles, taking sleep medications, engaging in screen time, exercising, shopping, daydreaming, reading, sleeping, flirting, playing video games, and pursuing adventurous sports. However, more extreme firefighter behaviors can include high-risk alcohol or drug use, disordered eating, self-injury, a chronic preoccupation with pornography, high-risk gambling, violence, and so on. Sometimes it takes more than one activity to soothe emotional pain, so a variety of firefighters get going at the same time. For example, an individual may combine alcohol or drug use with gambling and high-risk sexual practices. When these short-term solutions continue unabated, they accrue long-term costs, but firefighter parts are only concerned with immediate relief.

While firefighters are the agents of avoidance, they also provide hidden pleasures. These pleasures are not new. Historically, humans have always engaged in stimulating, relaxing, mood-altering, and transcendent practices. Pottery jars found in northern China that date from 7000 to 6600 BC provide early evidence of humans brewing alcohol. Ancient Sumerians in 3400 BC referred to the red poppy flower as the “joy plant.” Early evidence of cannabis and peyote date back at least 2,500 years. Gambling originated during the Paleolithic period, before written history, and the earliest six-sided pair of dice date to around 3000 BC in Mesopotamia. The pursuit of sexual pleasure is, of course, ubiquitous, although conventions about when, with whom, and how to be sexual have always varied widely.

In this way, firefighter parts have always played a vital role in our lives. In a balanced system, where they don’t need to spend every waking hour distracting from or soothing wounded parts, they can deploy their spontaneous, lively energy for other pursuits. In better circumstances, they counterbalance the drive of managers in good ways by shifting gears, changing the scenery, and promoting rest, relaxation, pleasure, fun, and novelty (Sykes, 2016). They offset the managerial focus on accomplishment with sensuous experiences—eating chocolate, enjoying the swig of a cold drink, racing downhill on a mountain bike, or napping on a still afternoon. When we listen closely to their intentions, even firefighters who are trapped in extreme roles want us to have pleasure, rest, and a change of perspective. They add spice and adventure as well rest and renewal to the humdrum security of a routine life.

COMMON BEHAVIORS OF FIREFIGHTER PARTS IN ADDICTIVE SYSTEMS

- Alcohol use
- Prescription or street drug use
- Disordered eating (bingeing, purging, comfort eating, or restricting)
- Gambling and overspending
- Sexual preoccupation, chronic sexual fantasizing, or sexual risk-taking

- Self-harm (cutting, head banging, etc.)
- Suicidal ideation or attempts
- Rage, violence, exploitation, or abuse of others
- Dissociating, tuning out, getting “lost,” or lacking awareness of the present
- Fantasizing about idealized images of success, power, or perfect relationships

COMMON FIREFIGHTER CHARACTERISTICS IN ADDICTIVE SYSTEMS

- They are chaotic, out of control, and driven to keep using
- They are both impulsive (unthinking and unconcerned about consequences) and compulsive (have no sense of choice)
- They are complex—can be either soothing or distracting in ways that are grossly overstimulating.
- They are heroic in that they don’t back down, even under fire; they will take a bullet for the system.
- They are narcissistic, self-absorbed, and concerned primarily with the emotions and unmet needs of certain parts in the client’s system.
- They are resistant to feedback and will avoid noticing or listening to the impact of their behavior on the client and others. In turn, they will also deny, hide evidence, and minimize (by lying) about how much the client is using.
- They are committed and will stay on the job until they’re convinced that exiles are safe enough.

COMMON FIREFIGHTER FEARS

- Fear that the hopelessness and despair of exiles will flood the system and cause a functional collapse
- Fear that managers will be exhaustingly shaming and inhibiting, and that they may evoke a suicidal part
- Fear that managers will accommodate other people too much
- Fear that the therapist, manager parts, family members, partners, and so on will control the client in detrimental ways
- Fear that their role as essential protectors will be underestimated
- Fear that they will be misunderstood



EXILES: EMOTIONALLY OVERWHELMING AND PAST-ORIENTED

Exiles, the third category of parts, are the young, sensitive, and most vulnerable parts of the inner system. When nurtured and protected, they embody innocence, trust in others, and open-heartedness. However, their dependence on others for nurture makes these tender parts vulnerable to wounding. They are quick to interpret the insult of neglect, or the pain of sexual, physical, or emotional transgressions, as information about their own inherent value. As a result, they feel lonely and become burdened with the belief that they are worthless, weak, undeserving, unlovable, or bad. Managers and firefighters fear these beliefs and feelings, and they work overtime to banish exiles—and their unintegrated memories and unhealed attachment wounds—from awareness. The extent to which protectors expend their energy on this work is proportional to the exile's wounding. The more severe the wound, the more energy protectors put into their job.

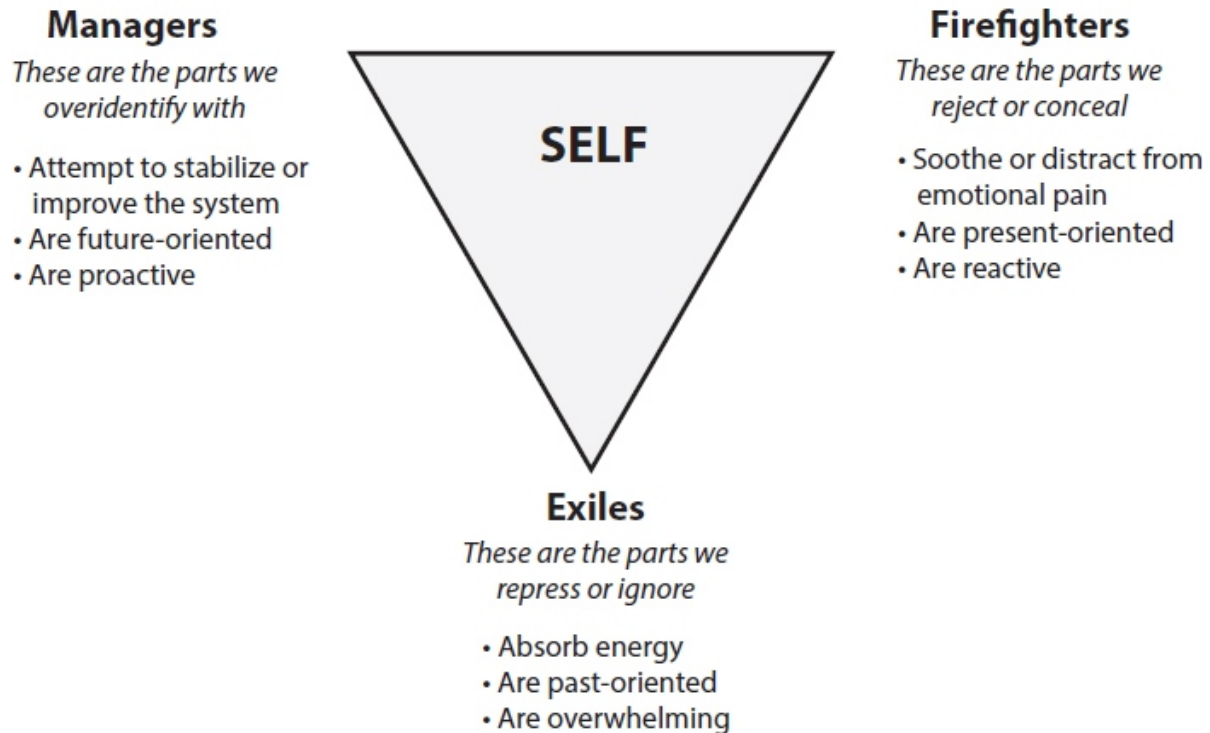
Simply having exiles does not necessarily mean our parents or caretakers were inattentive, careless, or negligent—no one can be perfectly attuned and available at all times. For this reason, we all have some wounded, exiled parts—and protectors to defend these parts. But the less supportive the environment, the greater the injury, and the more protectors feel obliged to exile injured parts. For example, children who were alienated from their family or community (e.g., a transgender or gay child in a transphobic, homophobic environment), who experienced significant loss (e.g., the death of a parent), who were neglected (e.g., lack of food, medical care, dental care, or housing), who were treated harshly (e.g., verbally, physically, or sexually assaulted), or who were exploited (e.g., made to provide primary care for siblings) are likely to develop extreme protectors at an early age. In fact, clients who grew up in rough circumstances often report having been drawn to shoplifting, smoking cigarettes, or eating for comfort by the age of six or eight. These were their first firefighter activities. The more alarming their situation, the harder their protectors worked to exile feelings that threatened to flood them and paralyze their internal systems.

This is only sensible. Who would not seek relief from a prolonged onslaught of emotional pain? When children aren't protected externally, they fashion internal protection from the tools at hand. This notion is borne out by the Adverse Childhood Experiences (ACE) study, which found that childhood trauma is associated with a variety of adverse health outcomes later in life (Felitti et al., 1998). The ACE study showed that people who were exposed to early trauma were likely to develop firefighter parts who self-medicated with nicotine, alcohol, drugs, comfort food—all while their manager parts were holding down jobs and trying to function.

In our view, it's never too late to heal wounded parts, even for clients who have gotten trapped in high-risk lifestyles that accrued big costs. Exiles may carry the great burden of a denigrated identity, but their natural state is the trusting, open-hearted, and playful nature of any well-nurtured baby or young child. When we honor and protect the vulnerability of these parts, we can help clients maintain access to a sense of wonder, joy, and lightheartedness. We can help them be carefree, open to new experiences, and sensitive to others without feeling overwhelmed or responsible for their troubles.

If IFS, we guide clients to rediscover their Self and release the exiles who were burdened with unbearable beliefs, like *I am too much* or *I am not enough*. We help them recognize that these beliefs are not factual information; they are a product of experience. For some, this is a long road filled with starts and stops. Yet the art of preventing the recurrence of addictive behaviors is the art of connecting with exiles safely, and this art is teachable.

The System: A Balancing Act*



SELF IS THE HEART OF THE MATTER

As we've discussed, the Self is our seat of kind awareness and wakeful presence. It is a compassionate, nonjudgmental witness to our experiences, inside and out. It is an inner place of knowing that can be obscured by the activities of parts but, like the sun, is always present and available when stormy weather clears. As clients embody their Self over the course of therapy, their awareness broadens to encompass both the external relational field and the inner landscape.

When we listen to a client's chaotic stories of self-destruction, we may wonder, *Does this person have a Self?* It's a good question. A client who regularly uses substances or engages in other addictive practices may have very little access to moments of insight or confidence. But their Self-energy does show up in more subtle ways, and an IFS therapist understands that all clients have the capacity to access their Self from the beginning of therapy, even if for only a few moments at a time. For example, the Self may appear in the simple wish for a less-fraught life, the urgent longing to be a better parent, or the desire to feel better when they wake up in the morning. Clients may also report hearing a wise inner voice when they're high or in an altered state of mind. It might warn them of danger, tell them to go home, or remind them to look after a friend.

During the course of therapy, we need to build on these small moments of clarity. We focus on identifying and dispersing the client's activated parts so they can experience at least a measure of inner calm, quiet, wisdom, and warmth. It can sometimes feel like we're moving at a snail's pace, moving forward only a centimeter or two at a time! Yet when parts get noticed and have the chance to connect with the Self, they loosen up, and this benefits the rest of the system. Critically, these new Self-to-part connections offer the client many moments of choice, each of which is an opportunity to try new behaviors.

BASIC CONCEPTS FOR THE CLINICAL APPLICATION OF IFS

Blending

To practice IFS, you must understand the concept of *blending*. When a client feels engulfed by a particular feeling or speaks from the perspective of just one part, we say that this part is *blended*. For example, let's say that a client drinks every day but declares, "There's no way my drinking is hurting anything!" The IFS therapist hears this as a firefighter part who loves or feels a need to drink. This part has strong feelings and a big impact, but it does not represent the client's best judgment or their system's majority view.

Similarly, let's say a client is struggling with binge eating and is despondent about their nightly eating pattern. They exclaim with disgust, "I'm terrible—this can't go on. I have to stop!" Here, the client is dismayed about eating compulsively but not curious about the function of the eating, which tells the IFS therapist that the client is blended with a judgmental manager as opposed to being connected with their Self.

Unblending

Parts can show up in one of three ways. First, they may appear as thoughts or ideas. For example, a client may think, *I now see that this job is not a good fit for me anymore*. Second, they can show up as a physical sensation or an urge. A client may experience a rapid heartbeat, a desire to get away, or a craving to eat, get high, gamble, or watch porn. Third, parts can manifest as a feeling or mood. For example, a client may say, "I've been worried all day" or "I feel so sad about my son." Since clients are usually not aware of blended parts—but, rather, identify with them—the therapist's primary role in IFS is to be a "parts detector" and guide the client in helping the part to unblend.

Toward that end, we help clients notice their parts, and then we guide them to persuade a target part that it would benefit by unblending. The therapist might say, "It sounds like we're hearing from a part of you who is really invested in drinking and can't see any problems. Is that right?" or "It sounds like you have one very critical part who is worried about the nighttime eating. Do you hear that part?" If the client is able to tune in to the part, the therapist can help them interview it about its intentions and fears.

Sometimes, clients are blended with several parts serially, so they float from one thought, feeling, or urge to the next—and these can be contradictory. For example, a client might begin a session with several parts pouring their hearts out: "I think I need to quit my job. It's not working for me anymore. I wanted to run out of there all week, but I can't afford to. And I'm so worried about what's happening with my daughter when I'm not at the house. I shouldn't have had so much to drink last night, but I can't sleep anymore, and then this morning I overslept and showed up late!"

When parts fight for center stage within consciousness, emotions mix and thoughts brawl. But whether one part is in the spotlight or several switch in and out, our goal is to make room for the client's Self, which happens when all their parts are willing to unblend. To that end, we ask the client to pick *one* target part, no matter how many parts the client describes at first. Then we ask, "How do you feel toward _____ [*the target part*]?" Their answer reveals either the presence of other blended parts or, if all parts have unblended, the presence of the client's Self.

If one part will not unblend, or many parts are blending in a serial fashion, we point out their conflict—for example, how one part wants to leave a certain job and another doesn't, or how one part wants to drink at night and another doesn't—and offer to help both sides. We ask if these parts would all be willing to unblend at least long enough to accept one moment of kindness and attention from the client's Self. Once they get connected in that way, we can ask which part needs the client's attention first. Just a few moments of everyone noticing each other can be revelatory and counts as unblending practice, with an aim of helping all parts unblend over time. When parts unblend, the client can see them more clearly, understand their desire for relief, and offer to help.

The Mini-Unblend for Quick Calming

Clients with addictive processes often show up for sessions feeling overwhelmed. Their emotions are running high, one chaotic story is spilling into another, and they are hungry for attention and care. This blast is likely to rouse the therapist's own managers, who want order, yet we also know that an abrupt push for clients' unblending may be received as unempathetic and dismissive. As a result, it's good to have another strategy for calming clients and slowing their systems down.

We recommend a *mini-unblend* (pause, breathe, notice), which welcomes the client's whole system, just as it is right now. Here's how to do it: We start with attuned, concerned listening and no agenda. We let the client spill all the information they need to. If it feels right, we make a mirroring statement or two, or ask a clarifying question. After a short while, we validate how hard the client has been trying, notice all they've been holding this week, or acknowledge all they had to do just to get here, and then suggest pausing to breath together. When the client calms, we can speak with the blended part directly, or the client can choose a target part for inquiry.

The 6 F's

Traditional talk therapy involves dialogue and a lot of guesswork. The client reports on their week, the therapist is empathic and asks insightful questions, the client responds, they ponder motives together, and so on. In IFS, instead of helping the client guess about their motives (*Maybe you binged because you were angry with your partner?*), we promptly move to nonjudgmental curiosity (*Let's find the part who binged last night and ask why*). If the bingeing part says it behaved this way because it didn't want the client to be angry, we understand the behavioral sequence, and we can then help the client build relationships with both the bingeing part and the angry part—and we can move toward learning whom they protect. In this way, the client repeatedly discovers that protective parts shield vulnerable parts, which naturally leads to the key question, *Would you need to act this way if we could help the vulnerable part?*

We conduct nonjudgmental inquiries by starting with a chain of six questions—which we call the 6 F's—and continue on from there with more questions.

Clients are generally surprised and pleased when their parts talk back (or somehow communicate with them) in response to these questions. Hearing directly from their parts about their fears and their disagreements with each other helps the client make sense of subjective experiences that otherwise seem chaotic, confusing, and nonsensical. This, alone, can be a great relief. Since parts are less likely to judge what they understand, critical managers may also calm down or unblend more easily when they listen to the client interview reactive firefighters. Every little step on this inner journey moves in the direction of parts cooperating with each other and trusting the client's Self.

THE 6 F'S

1. **Find** a part: Who needs your attention today?
2. **Focus** on the part: How does it show up? What kind of energy does it have? Where do you notice it in or around your body?
3. How do you **feel** toward the part? Accepting, judgmental, or neutral? When you notice this part, do you feel open and curious, guarded and cautious, angry and rejecting, or something else?
4. **Flesh** out the part's role: What are its original intentions for the internal system? What are its signature behaviors? How long has it been in this role? Who is it protecting?

5. **Befriend** the part: How does it respond to you? What would it need to trust the connection with you?
 - If the part feels safe enough with the client to share without reservation, we can be confident that the client's Self is present. But if a protector is criticizing the target part or doesn't want to listen to it, we know a critical manager is present, and we ask it to unblend. If it refuses, it becomes our target part.
6. Discover the part's **fears**. What would other protectors do if this one stopped its behavior? Has that scenario played out in the past? What would happen to the hurt, exiled parts if it stopped this behavior? Has that ever happened in the past?

Client Exercise**Getting to Know a Manager**

Managers are proactive and future-oriented parts who want you to improve and make progress. Toward this end, they may engage in a variety of behaviors like perfectionism, self-criticism, analyzing, approval-seeking, planning, striving, minimizing, controlling, moralizing, and overworking. This exercise guides you to befriend a manager part whose activity feels familiar to you. Choose any part who would like your attention; it doesn't need to be extreme.

1. First, get comfortable. There is no right way to do this exercise. Just follow your breath to your inner world, and when you're ready, ask for a manager part who would like some attention.
2. Once you've identified a manager, place that part on the other side of a oneway mirror. Invite it to be active and do what it usually does while you observe. Notice its body language and facial expressions. Listen to what it says. For example, you may begin to observe a part who is always planning: when to shop for groceries, what time to retrieve the children, when to walk the dog, how to fit in extra tasks, how to complete a work project, and so on. Tell that part that you're right there watching and you want to know more. If you don't see the part because it's more auditory than physical—for example, a perfectionist part who shows up as a voice in your mind or as a buzzing pressure near your head—you can still let it know that you're listening or feeling its energy and you're open to its message.
3. If at any point another part activates, ask it to relax and offer to check on it later, then return to the manager who is your target part. Whenever reactive parts emerge in this manner, ask them to relax. When you feel open and curious, return to the target manager and resume listening. Notice how you feel toward this manager, and if you're still feeling curious, here are some questions to ask:
 - When you pressure me to _____ [*fill in the blank with the manager's usual behavior, such as planning ahead, making no mistakes, making sure my supervisor is happy, etc.*], how are you trying to help?
 - How long have you been helping me this way?
 - How old are you?

- Do you remember a time when acting like this became especially important? What was going on then?
- Was there ever a time when you couldn't help me to act this way? If so, what was that like for you and what happened?
- What are you afraid would happen if you took a break or stopped doing what you do? How long have you had these fears and concerns?
- Have you ever been in a situation where these fears actually came true?
- Have you ever tried to take a break from doing what you do? What happened then? How did you return to your role and get things back on track?
- When you look at me, whom do you see? (Sometimes the part sees another part. For example, it might see you as an eight-year-old child or as an acting-out firefighter part.)
- What else do you want me to know about you?

As you ask these questions, you might find that some manager parts use language to talk back to you, as if you are conversing. Other parts communicate nonverbally by showing you images to answer your questions or explain their motivations without using words. Whatever communication you get from the part, check to see if your heart is open. If so, let it know that you're paying attention and that you understand what it's telling you. If you don't understand, ask it to clarify. Some people find it helpful to respond to the part with words like, "Now that makes sense to me. I get why you want to be sure I don't fail. I appreciate your hard work."

When you understand the part's intentions and fears, thank it for sharing and acknowledge its hard work. Let it know that you want to stay in touch and help out so it's not alone. If it doesn't seem to know or trust you, acknowledge that this can easily be the case because relationships take time; let it know that you're prepared to be patient. Finally, notice how the part responds to your support, offer to stay connected, and take notes for future reference.

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Client Exercise

Getting to Know a Firefighter

Firefighters are protectors who react to manager overreach and to the surfacing of exiled pain. The most common firefighter activities involve self-medicating with alcohol, drugs, food, or sexual practices, including looking at pornography. Other firefighter parts also use gambling, shopping, screen time, exercise, physical violence, and self-harm behaviors, to name a few. Many people combine firefighter options (e.g., drinking and engaging in unprotected sex). However, this exercise guides you to befriend *one* firefighter part whose activity feels familiar. The part's behavior doesn't need to be extreme, and it can be a former behavior, like smoking in high school.

1. First, get comfortable. There is no right way to do this exercise. Just follow your breath inside, and when you're ready, ask who would like your attention.
2. Once you've identified a firefighter, place that part on the far side of a one-way mirror and invite it to be active and do what it usually does while you observe. Notice its body language and facial expressions. Is it happy and relaxed? Tense and over-stimulated? Drowsy and checked out?
3. How do you feel as you watch this part engage in its usual practice? If you notice judgment, fear, or embarrassment coming up, ask those parts to step back and offer to check on them later. Tell them that you (the Self) are merely connecting with this part, not encouraging or endorsing its behaviors. When you feel open and curious, return to the target firefighter part and ask these questions:
 - When you pressure me to _____ [*fill in the blank with the firefighter's usual behavior, such as having a few beers or bingeing on late-night movies*], how are you trying to help?
 - How long have you been helping me this way?
 - How old are you?
 - Do you remember a time when acting like this became especially important? What was going on then?
 - Was there ever a time when you couldn't help me by acting this way? If so, what was that like for you and what happened?

- What are you afraid would happen if you took a break or stopped doing what you do?
How long have you had these fears and concerns?
- Have you ever been in a situation where these fears actually came true?
- Have you ever tried to take a break from doing what you do? What happened then? How did you return to your role?
- When you look at me, whom do you see? (Sometimes the part sees another part. For example, it might see you as an eight-year-old child, or as a critical manager.)
- What else do you want me to know about you?

As you ask these questions, you might find that some firefighter parts use language to talk back to you, as if you are conversing. Other parts communicate nonverbally by showing you images to answer your questions or explain their motivations without using words. Whatever communication you get from the part, check to see if your heart is open. If so, let it know that you're paying attention and that you understand what it's telling you. If you don't understand, ask it to clarify. Some people find it helpful to respond to the part with words like, "Now that makes sense to me. I get why you want to numb me from these painful emotions. I appreciate your hard work."

When you understand the part's intentions and fears, thank it for sharing and acknowledge its hard work. Let it know that you want to stay in touch and help out so it's not alone. If it doesn't seem to know or trust you, acknowledge that this can easily be the case because relationships take time; let it know that you're prepared to be patient. Finally, notice how the part responds to your support, offer to stay connected, and take notes for future reference.

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Direct Access and In-Sight

Clients will nearly always arrive to their first appointment blended with a part. If they are concerned and hopeful, they're blended with a manager who convinced them to try therapy. If they're wary, hostile, paranoid, or accusing, they're blended with a firefighter. If they're hopeless and tearful, they're blended with an exile. We start by interviewing the blended part directly, and we have a couple of choices about how to do that:

1. The first choice is to act as if we're speaking with the client, not just a part of the client, even though we know we are speaking with a part. In this case, we use the pronoun *you*—what do *you* notice, what do *you* feel, what brings *you* here, and so on. This sounds just like traditional talk therapy, and we call it *implicit direct access*.
2. The second choice is to name the part as a part and talk with it about the client. In this case, we use the client's preferred pronouns to talk about them in the third person. We call this *explicit direct access*. We might say, for example, "So you are the part of Mark who is concerned about his cocaine use. Is that right?"

Once we speak directly with one or more of the client's blended parts using direct access, we then ask the part to unblend. If it won't unblend, we assume it has a good reason and we keep using direct access to learn more. But if it will unblend, the client's Self shows up and the internal Self-to-part attachments we are looking for begin to crystallize. At this point, it's best for the client's Self to take over and communicate directly with the client's parts, which is known as *in-sight*. If the client has steady access to their Self, the therapist can just step back and ask, "What needs to happen next?" But if the client's access to the Self is sketchy, the therapist can keep offering guidance as needed. As illustrated in examples throughout this manual, both *in-sight* and direct access are effective, though direct access can be particularly useful with polarized parts. Use both strategies liberally across sessions and clients.

What If?

Once proactive (manager) and reactive (firefighter) teams notice the client's Self and understand that they share the goal of protecting injured parts, we can suggest a new way forward by asking the following series of *what-if* questions. Our vision is hopeful (*There is an easier way!*), and protectors are typically intrigued by optimistic hypotheticals.

HYPOTHETICAL WHAT-IFS TO HELP A PART UNBLEND

- What if there was another way to handle this vulnerability?
- What if there was an effective way out of this chaotic, addictive cycle?
- What if isolated, wounded parts could heal in relationship with the Self?
- What if we could help the firefighters so they didn't have to handle the vulnerable parts all alone?

- What if we could help the managers so they didn't have to shoulder all the responsibility by themselves?
- What if you had support?

After the client has reflected on one or more of these what-ifs, ask them to pose the following question to their manager and firefighter teams: Would you be open to taking a break if you saw the Self helping these vulnerable parts?

* Adapted with permission from “An IFS Lens on Addiction: Compassion for Extreme Parts,” by C. Sykes, in M. Sweezy & E. L. Ziskind (Eds.), *Innovations and Elaborations in Internal Family Systems Therapy* (p. 30), 2017, Routledge (<https://doi.org/10.4324/9781315775784>). Copyright 2017 by Cece Sykes, LCSW.